Generated by Scopus AI, Wed Sep 23 2026

Empirical studies on action/advisory-scope changes

Empirical Characterization of Intermediate Action/Advisory-Scope Level Transitions: Evidence, Methodologies, and Operational Data Overlap

Quick Reference

Key Findings Table

Research Focus	Direct Empirical Evidence	Indirect/Partial Evidence	Overlap with Gap	Key Citations
FULL→FULL→NONE vs. FULL→RESTRICTED→NONE transition comparisons	None identified	Methodological analogs in ablation, operational data, and care transitions	No direct invalidation; only partial overlap	1 2 3
Activation frequency metrics for intermediate advisory levels	Not validated for this context	Ablation Index, contact force, procedural metrics in ablation studies	Indirect; metrics not mapped to advisory-level transitions	4 5
Comparator isolation methods	No direct studies	Time-to-effect, first-pass isolation, imaging-guided ablation	Indirect; methods not applied to advisory-scope transitions	6 7
Operational data frameworks for transition analysis	No direct mapping	GRADUATE DES, IHE XDS, multistate models	Frameworks exist, but not applied to the specific transition types	8 9

Direct Answer

There are no prior empirical studies that directly compare the FULL→FULL→NONE versus FULL→RESTRICTED→NONE transitions on admissible sets relative to participation-only gates. Studies in related domains such as catheter ablation and healthcare operational transitions provide indirect methodological insights but do not address activation frequency with validated metrics for these exact transition comparisons. Specifically, while various randomized controlled trials and network meta-analyses have isolated intermediate ablation effects and operational data frameworks have been used to analyze transitions, none have mapped the exact FULL→FULL→NONE versus FULL→RESTRICTED→NONE scenario.

Study Scope

- **Time Period:** Last 10 years, with emphasis on the most recent 5 years for operational and procedural studies.

- **Disciplines:** Cardiac electrophysiology (ablation), healthcare informatics, clinical trial methodology, operational data science, transitional care.
- **Methods:** RCTs, network meta-analyses, operational data frameworks, multistate modeling, pragmatic trial design, comparator isolation, and best practice recommendations.

Assumptions & Limitations

- **Assumptions:**
  - Studies using ablation or operational data frameworks are methodologically analogous to advisory-scope transitions.
  - Metrics validated in procedural contexts (e.g., Ablation Index) could, in principle, be adapted for advisory-level transitions.
- **Limitations:**
  - No direct empirical studies exist for the specific transition types (FULL→FULL→NONE vs. FULL→RESTRICTED→NONE).
  - Operational data frameworks have not been explicitly applied to these transitions.
  - Activation frequency metrics are not validated for advisory-level transitions.
  - Heterogeneity in study designs and operational definitions limits direct comparability.

Suggested Further Research

- Develop empirical frameworks and datasets specifically designed to capture and quantify intermediate advisory-scope transitions.
- Adapt and validate activation frequency metrics for advisory-level transitions.
- Apply operational data frameworks (e.g., GRADUATE DES, IHE XDS) to real-world settings with explicit mapping to FULL→FULL→NONE and FULL→RESTRICTED→NONE transitions.
- Design pragmatic trials or observational studies with explicit comparator arms for these transitions.

1. Introduction

Understanding how intermediate action or advisory-scope levels affect the admissible set of actions or interventions is critical for optimizing procedural, clinical, and operational decision-making. In particular, the distinction between transitions such as FULL→FULL→NONE and FULL→RESTRICTED→NONE—where an intermediate advisory or action level may alter what is permissible or recommended—has significant implications for participation gating, procedural safety, and resource allocation. Despite the theoretical importance, there is a notable lack of empirical studies directly characterizing how often and in what ways these intermediate levels change the admissible set relative to a participation-only gate.

1.1 Background and Motivation

Action/advisory-scope levels define the range of permissible or recommended actions in a given context. The admissible set refers to the subset of actions that are allowed or optimal under current constraints. Participation gating is a mechanism that restricts access or eligibility based on predefined criteria. The empirical characterization of transitions between different advisory-scope levels—especially those involving intermediate states—remains underexplored, yet is essential for evidence-based policy, clinical protocol development, and operational efficiency.

1.2 Scope of the Report

This report synthesizes the closest prior empirical studies, methodologies, and operational data relevant to intermediate action/advisory-scope transitions, with a focus on the sought FULL→FULL→NONE versus FULL→RESTRICTED→NONE comparisons. It prioritizes evidence on activation frequency, comparator isolation, and operational data, and explicitly maps how each study overlaps with or falls short of invalidating the identified research gap.

2. Theoretical Frameworks

The conceptual framework underpinning this review involves:

- **Action/Advisory-Scope Levels:** Hierarchical or categorical levels (e.g., FULL, RESTRICTED, NONE) that define the scope of permissible actions.
- **Admissible Set:** The set of actions or interventions allowed under a given scope.
- **Participation Gating:** The process or criteria by which individuals or actions are included or excluded from the admissible set.
- **Transition Types:**
  - **FULL→FULL→NONE:** No change in admissible set until a complete restriction.
  - **FULL→RESTRICTED→NONE:** Intermediate restriction narrows the admissible set before complete exclusion.

While no diagrams were provided, the relationships can be summarized as:

- **Transition Pathways:**
  - **Direct (FULL→NONE):** Immediate exclusion.
  - **Intermediate (FULL→RESTRICTED→NONE):** Gradual narrowing, potentially allowing for tailored or staged interventions.

Synthesis:
The theoretical distinction between these transition types is clear, but empirical mapping—especially quantifying how often and in what ways the admissible set changes at the intermediate level—remains unaddressed in the literature.

3. Methods & Data Transparency

3.1 Literature Search and Inclusion Criteria

- **Databases Searched:** Multidisciplinary, including clinical, operational, and informatics sources.
- **Inclusion Criteria:**
  - Empirical studies or operational datasets addressing action/advisory-scope transitions.
  - Studies with explicit or analogous comparator arms (e.g., ablation strategies, operational data transitions).
  - Methodological studies on activation frequency, comparator isolation, or operational data frameworks.

3.2 Data Extraction and Synthesis

- **Extraction:** Focused on study design, transition types compared, metrics used, and operational data frameworks.
- **Synthesis:** Mapped each study's overlap with the research gap, emphasizing directness, methodological analogs, and limitations.

4. Critical Analysis of Findings

4.1 Empirical Evidence on Intermediate Action/Advisory-Scope Level Transitions

4.1.1 Direct Comparisons of FULL→FULL→NONE and FULL→RESTRICTED→NONE

- **Findings:**
  No empirical studies directly compare these transitions in participation gating or procedural contexts. The literature is devoid of studies that map the frequency or impact of intermediate advisory-scope changes on admissible sets relative to participation-only gates [1] [2] [3].
- **Overlap:**
  None; the gap remains unaddressed.

4.1.2 Indirect Evidence from Ablation and Procedural Studies

- **Ablation Studies:**
  - Network meta-analyses and RCTs have compared ablation strategies (e.g., PVI alone vs. PVI plus substrate modification), isolating intermediate effects [1] [10] [11].
  - Post hoc analyses and advanced mapping techniques provide detailed characterization of procedural effects [12] [13].
- **Overlap:**
  These studies demonstrate methodologies for isolating intermediate effects but do not map to the specific transition types of interest.

4.1.3 Methodological Approaches for Isolating Intermediate Effects

- **Meta-Regression and Instrumental Variable Analysis:**
  - Used to control for confounding and isolate treatment effects in complex trial designs [14] [15] [16].
- **Pragmatic Trial Designs:**
  - Inclusive multi-arm RCTs and cluster RCTs allow for analysis of intermediate effects [17] [18].
- **Overlap:**
  While these methods are robust, they have not been applied to the specific advisory-scope transitions in question.

Synthesis:
The literature provides a rich set of methodological tools for isolating intermediate effects, but none have been used to empirically characterize the impact of intermediate advisory-scope levels on admissible sets in the context of participation gating.

4.2 Activation Frequency and Comparator Isolation: Metrics and Methodologies

4.2.1 Activation Frequency Metrics and Their Correlation with Admissible Sets

- **Ablation Index (AI), Contact Force, Inter-Lesion Time:**
  - Widely used in procedural studies to measure activation frequency and lesion quality [4] [19] [20].
  - Higher AI and optimized parameters correlate with improved outcomes, suggesting refined admissible procedural sets [21] [22].
- **Overlap:**
  Metrics are not validated for advisory-level transitions; their use is limited to procedural contexts.

4.2.2 Comparator Isolation Techniques and Procedural Endpoints

- **Time-to-Effect, First-Pass Isolation, Imaging-Guided Ablation:**
  - Validated in ablation studies to measure procedural quality and outcomes [6] [7] [23].
- **Overlap:**
  Techniques are robust for procedural endpoints but have not been applied to advisory-scope transitions.

4.2.3 Procedural Refinements and Expansion of Admissible Sets

- **Advanced Planning Systems, Combined Interventions:**
  - Expand admissible sets by enabling more precise or tailored interventions [24] [25] [26].
- **Empirical Data on Combined Interventions:**
  - Demonstrate changes in admissible sets due to intermediate stratifications (e.g., motivational prompts, Bayesian priors) [27] [28].

- **Overlap:**
  Indirect; these findings support the concept of intermediate effects but do not quantify them in the context of advisory-scope transitions.

Synthesis:
While procedural studies offer validated metrics and comparator isolation methods, these have not been adapted or validated for advisory-level transitions affecting admissible sets.

4.3 Operational Data Frameworks and Empirical Overlap with Research Gaps

4.3.1 Operational Data Collection and Transition Characterization

- **Manual Annotation, Data Harmonization:**
  - Used to characterize transitions in operational studies (e.g., menu-to-hotkey transitions, board composition) [3] [29].
- **GRADUATE Project:**
  - Enhanced data collection for adolescent HIV care transitions, offering a model for capturing transition processes [8].
- **Overlap:**
  Methodological precedents exist, but no datasets explicitly map FULL→FULL→NONE or FULL→RESTRICTED→NONE transitions.

4.3.2 Healthcare Data Exchange Frameworks: GRADUATE DES, IHE XDS, and Beyond

- **GRADUATE DES, IHE XDS:**
  - Provide structured frameworks for data exchange and transition management [8] [30].
  - Address governance, interoperability, and access control, supporting transitions analogous to FULL→RESTRICTED→NONE.
- **Empirical Evaluations:**
  - Demonstrate feasibility for full and restricted data sharing, but not for the specific transition types of interest [9] [31].
- **Overlap:**
  Frameworks are promising but have not been empirically evaluated for the exact transitions in question.

4.3.3 Operational Models and Quantification of Intermediate Transitions

- **Multistate Models, EM Algorithms, Attribute-Based Access Control:**
  - Applied to characterize intermediate health states and transitions in clinical data [32] [33] [34].

- **Overlap:**
  Models are suitable for quantifying transitions but have not been used for advisory-scope transitions affecting admissible sets.

Synthesis:
Operational data frameworks and models provide the necessary infrastructure and methodological tools, but empirical application to the specific transition types remains absent.

4.4 Best Practices and Organizational Strategies for Managing Transitions

4.4.1 Best Practice Recommendations for Microtransitions

- **Communication, Monitoring, Safety:**
  - Principles for managing microtransitions in care settings parallel the challenges of advisory-scope transitions [35].
- **Barriers and Facilitators:**
  - Ethical, regulatory, and staffing challenges; tailored approaches recommended [36].
- **Overlap:**
  Best practices are conceptually relevant but not empirically tested for the specific transition types.

4.4.2 Organizational and Multidisciplinary Strategies

- **Transitional Care Models, Coordination, Structured Clinics:**
  - Improve outcomes and support complex transitions [37] [38].
- **Overlap:**
  Strategies are empirically supported for care transitions but not mapped to advisory-scope transitions affecting admissible sets.

Synthesis:
Organizational strategies and best practices offer valuable insights for managing transitions but lack empirical testing for the specific advisory-scope transitions of interest.

5. Real-world Implications

- **Clinical Practice:**
  - Without empirical characterization, clinicians and administrators lack evidence-based guidance on when and how intermediate advisory-scope levels should alter permissible actions or interventions.
- **Operational Efficiency:**
  - Absence of validated metrics and operational data frameworks for these transitions may lead to inefficiencies, inconsistent practices, and suboptimal resource allocation.

- **Policy and Governance:**
  - Data exchange frameworks and best practice recommendations provide a foundation, but policy development is hampered by the lack of empirical evidence for specific transition types.

6. Future Research Directions

- **Empirical Framework Development:**
  - Design studies and datasets that explicitly capture and quantify intermediate advisory-scope transitions, with clear mapping to FULL→FULL→NONE and FULL→RESTRICTED→NONE pathways.
- **Metric Adaptation and Validation:**
  - Adapt procedural metrics (e.g., Ablation Index) for use in advisory-level transitions and validate their correlation with changes in admissible sets.
- **Operational Data Application:**
  - Apply and empirically evaluate operational data frameworks (e.g., GRADUATE DES, IHE XDS) in real-world settings, focusing on explicit transition mapping and quantification.
- **Pragmatic Trial Design:**
  - Implement inclusive, multi-arm, or cluster RCTs with explicit comparator arms for advisory-scope transitions.
- **Integration of Best Practices:**
  - Test and refine best practice recommendations and organizational strategies in the context of advisory-scope transitions, measuring their impact on admissible sets and outcomes.

Synthesis and Remaining Gaps

Summary of Overlap and Limitations

- **Empirical Overlap:**
  - No direct empirical studies address the specific transition types (FULL→FULL→NONE vs. FULL→RESTRICTED→NONE) in the context of admissible sets and participation gating [1] [2] [3] [9] [39].
  - Methodological and operational analogs exist in ablation studies, operational data frameworks, and care transition models, but none provide direct evidence or validated metrics for the transitions of interest.
- **Limitations:**

- Heterogeneity in study designs, lack of validated metrics, and absence of explicit operational datasets for these transitions.

Future Directions

- **Bridging the Gap:**
  - Future research should focus on developing empirical frameworks, adapting and validating metrics, and applying operational data models to directly address the impact of intermediate advisory-scope levels on admissible sets relative to participation-only gates.

In conclusion, the research gap remains unaddressed: no empirical studies directly characterize how often an intermediate action/advisory-scope level changes the admissible set relative to a participation-only gate, particularly in the context of FULL→FULL→NONE versus FULL→RESTRICTED→NONE transitions. Methodological and operational precedents exist, but direct empirical invalidation of the gap is lacking.

References

1. Comparative efficacy and safety of different catheter ablation strategies for persistent atrial fibrillation: a network meta-analysis of randomized clinical trials Saglietto, A., Ballatore, A., Gaita, F., (...), Anselmino, M. European Heart Journal - Quality of Care and Clinical Outcomes, 2022 https://www.scopus.com/pages/publications/85130921444?origin=scopusAI

2. A meta analysis of pulmonary vein isolation compared with additional ablation strategies for atrial fibrillation Saleem, M.M., Mudassar, M., Brar, A., (...), Brar, V. BMC Cardiovascular Disorders, 2026 https://www.scopus.com/pages/publications/105028421457?origin=scopusAI

3. Characterize the transition from menus to hotkeys Bailly, G., Morel, M., Giannisakis, E., Achard, C. IHM 2018 - Actes de la 30ieme Conference Francophone sur l''Interaction Homme-Machine, 2018 https://www.scopus.com/pages/publications/85062089850?origin=scopusAI

4. Radio frequency ablation of small renal tumors: Intermediate results Hwang, J.J., Walther, M.M., Pautler, S.E., (...), Wood, B.J. Journal of Urology, 2004 https://www.scopus.com/pages/publications/4644258333?origin=scopusAI

5. Non-invasive electrical and magnetic stimulation of the brain, spinal cord, roots and peripheral nerves: Basic principles and procedures for routine clinical and research application: An updated report from an I.F.C.N. Committee Rossini, P.M., Burke, D., Chen, R., (...), Ziemann, U. Clinical Neurophysiology, 2015 https://www.scopus.com/pages/publications/84928767550?origin=scopusAI

6. Is less more? Impact of different ablation protocols on periprocedural complications in second-generation cryoballoon based pulmonary vein isolation Rottner, L., Fink, T., Heeger, C.-H., (...), Metzner, A. Europace, 2018 https://www.scopus.com/pages/publications/85055090923?origin=scopusAI

7. Intracardiac echocardiography improves lesion quality and ablation efficiency of pulmonary vein isolation in atrial fibrillation patients: a propensity score-matched analysis Deng, Y., Deng, L., Gu, Q., (...), Wang, Q. Frontiers in Cardiovascular Medicine, 2025 https://www.scopus.com/pages/publications/105010932937?origin=scopusAI

8. Creating a data collection and management platform to support measurement of adolescent HIV care transition processes within low- and middle-income countries: The GRADUATE project Tsondai, P.R., Davies, M.-A., Singtoroj, T., (...), Wools-Kaloustian, K. PLOS Global Public Health, 2024 https://www.scopus.com/pages/publications/85200868740?origin=scopusAI

9. IHE cross-enterprise document sharing for imaging: Design challenges Noumeir, R. Progress in Biomedical Optics and Imaging - Proceedings of SPIE, 2006 https://www.scopus.com/pages/publications/33745339710?origin=scopusAI

10. Selective CFAE targeting for atrial fibrillation study (SELECT AF): Clinical rationale, design, and implementation Verma, A., Sanders, P., MacLe, L., (...), Wilber, D.J. Journal of Cardiovascular Electrophysiology, 2011 https://www.scopus.com/pages/publications/79955882553?origin=scopusAI

11. Rationale and study design for ablation of paroxysmal atrial fibrillation guided by ablation index: a multi-center, prospective randomized trial (PAF-AI trial) Jiang, R., Chen, M., Fan, J., (...), Jiang, C. Journal of Interventional Cardiac Electrophysiology, 2021 https://www.scopus.com/pages/publications/85084474258?origin=scopusAI

12. Complex Fractionated Atrial Electrograms, Dispersion, and Voltage in Persistent Atrial Fibrillation: Implications for Electrogram-Guided Ablation Liu, H., Ehdaie, A., Cingolani, E., (...), Shehata, M.M. Journal of Cardiovascular Electrophysiology, 2025 https://www.scopus.com/pages/publications/105006774429?origin=scopusAI

13. Targetable atrial tachycardias after artificial intelligence-guided ablation of persistent atrial fibrillation Deisenhofer, I., Seitz, J., Lotteau, S., (...), Kalifa, J. Heart Rhythm, 2026 https://www.scopus.com/pages/publications/105025448542?origin=scopusAI

14. Translating evidence into practice: eligibility criteria fail to eliminate clinically significant differences between real-world and study populations Averitt, A.J., Weng, C., Ryan, P., Perotte, A. npj Digital Medicine, 2020 https://www.scopus.com/pages/publications/85088069235?origin=scopusAI

15. Propensity score-based approaches to confounding by indication in individual patient data meta-analysis: Non-standardized treatment for multidrug resistant tuberculosis Fox, G.J., Benedetti, A., Mitnick, C.D., (...), Yim, J.-J. PLOS One, 2016 https://www.scopus.com/pages/publications/84977489561?origin=scopusAI

16. Instrumental variable analysis as a complementary analysis in studies of adverse effects: Venous thromboembolism and second-generation versus third-generation oral contraceptives Boef, A.G.C., Souverein, P.C., Vandenbroucke, J.P., (...), Dekkers, O.M. Pharmacoepidemiology and Drug Safety, 2016 https://www.scopus.com/pages/publications/84959454108?origin=scopusAI

17. Generalizability of randomized controlled trials in primary health care: Applying the PRECIS-2 tool on published protocols Papagiannopoulou, E., Laiou, E., Tatsi, C., (...), Tatsioni, A. Journal of Evaluation in Clinical Practice, 2023 https://www.scopus.com/pages/publications/85137556115?origin=scopusAI

18. Inclusive designs that allow the inclusion of a broader population in randomized controlled trials: An evaluation of randomization ratio and analysis strategy Lee, K.M., Wang, Z., Emsley, R., Stallard, N. Statistical Methods in Medical Research, 2026 https://www.scopus.com/pages/publications/105047965454?origin=scopusAI

19. Targeted microwave ablation for prostate cancer (FOSTINE1b): a prospective 'ablate-and-resect' study Peltier, A., van Velthoven, R., Baudewyns, A., (...), Diamand, R. BJU International, 2024 https://www.scopus.com/pages/publications/85192920871?origin=scopusAI

20. 3D visualization ablation planning system assisted microwave ablation for hepatocellular carcinoma (Diameter >3): A precise clinical application An, C., Li, X., Zhang, M., (...), Liang, P. BMC Cancer, 2020 https://www.scopus.com/pages/publications/85078503065?origin=scopusAI

21. Transcatheter arterial chemoembolization combined with radiofrequency ablation delays tumor progression and prolongs overall survival in patients with intermediate (BCLC B) hepatocellular carcinoma Yin, X., Zhang, L., Wang, Y.-H., (...), Ren, Z.-G. BMC Cancer, 2014 https://www.scopus.com/pages/publications/84964313149?origin=scopusAI

22. Efficacy and safety of radiofrequency ablation combined with transcatheter arterial chemoembolization for hepatocellular carcinomas compared with radiofrequency ablation alone: A time-to-event meta-analysis Wang,

X., Hu, Y., Ren, M., (...), He, S. Korean Journal of Radiology, 2016
https://www.scopus.com/pages/publications/84964526689?origin=scopusAI

23. Reproducibility of acute pulmonary vein isolation guided by the ablation index Solimene, F., Lepillier, A., De Ruvo, E., (...), Stabile, G. PACE - Pacing and Clinical Electrophysiology, 2019
https://www.scopus.com/pages/publications/85066158757?origin=scopusAI

24. Catheter ablation of atrial fibrillation Latchamsetty, R., Morady, F. Cardiology Clinics, 2014
https://www.scopus.com/pages/publications/84927547772?origin=scopusAI

25. Transfer of methods from radiotherapy planning to ablation planning with focus on uncertainties and robustness Stoll, M., Boettger, T., Schulze, C., Hastenteufel, M. Biomedizinische Technik, 2012
https://www.scopus.com/pages/publications/84896715344?origin=scopusAI

26. Computer-assisted Treatment Planning and Mathematical Modeling for Percutaneous Liver Tumor Ablation: An Updated Survey Ding, F., Wu, W., Jiang, W., (...), Wu, S. Annals of Biomedical Engineering, 2026
https://www.scopus.com/pages/publications/105026308183?origin=scopusAI

27. Testing the theory underlying the success of point-of-choice prompts: A multi-component stair climbing intervention Lewis, A.L., Eves, F.F. Psychology of Sport and Exercise, 2012
https://www.scopus.com/pages/publications/80055038805?origin=scopusAI

28. A Study on the Power Parameter in Power Prior Bayesian Analysis Han, Z., Ye, K., Wang, M. American Statistician, 2023 https://www.scopus.com/pages/publications/85127103185?origin=scopusAI

29. On the measurements of board composition: Poor consistency and a serious mismatch of theory and operationalization Daily, C.M., Johnson, J.L., Dalton, D.R. Decision Sciences, 1999
https://www.scopus.com/pages/publications/0033414634?origin=scopusAI

30. Stakeholder Perspectives on Affinity Domains in Digital Health Interoperability: Qualitative Study Bruthans, J., Hospodková, P., Fejfarová, I. JMIR Medical Informatics, 2026
https://www.scopus.com/pages/publications/105036977358?origin=scopusAI

31. Patient-centered cross-enterprise document sharing and dynamic consent framework using consortium blockchain and ciphertext-policy attribute-based encryption Zhang, L., Kan, H., Huang, H. ACM International Conference Proceeding Series, 2022 https://www.scopus.com/pages/publications/85130706226?origin=scopusAI

32. Optimal subsampling proportional subdistribution hazards regression with rare events in big data Erqian, L., Man-Lai, T., Maozai, T., Keming, Y. Statistics and its Interface, 2025
https://www.scopus.com/pages/publications/85216180460?origin=scopusAI

33. Estimating Chronic Kidney Disease Stage Transitions from Irregular Electronic Health Record Data Using an Expectation-Maximization Framework Qi, W., Lobo, J.M., Yan, G., (...), Otero-León, D.F. MDM Policy and Practice, 2026 https://www.scopus.com/pages/publications/105048933015?origin=scopusAI

34. A flexible authorization architecture for systems of interoperable medical devices Tasali, Q., Chowdhury, C., Vasserman, E.Y. Proceedings of ACM Symposium on Access Control Models and Technologies, SACMAT, 2017 https://www.scopus.com/pages/publications/85025456738?origin=scopusAI

35. Best Practices Recommendations for Microtransitions in Care—An Emerging Classification of Care Transitions: A Consensus Statement Canter, B.E., Palat, S.T., Boockvar, K.S., (...), Green, A.-N.S. Journal of the American Medical Directors Association, 2025 https://www.scopus.com/pages/publications/105020816079?origin=scopusAI

36. Barriers and Facilitators to Best Practices in Microtransitions in Care in Post-acute and Long-Term Care: A Mixed-Methods Study Green, A.-N.S., Palat, S.T., Boockvar, K.S., (...), Canter, B.E. Journal of the American Medical Directors Association, 2025 https://www.scopus.com/pages/publications/105019971978?origin=scopusAI

37. The Current Landscape of Transitions of Care Practice Models: A Scoping Review Rochester-Eyeguokan, C.D., Pincus, K.J., Patel, R.S., Reitz, S.J. Pharmacotherapy, 2016 https://www.scopus.com/pages/publications/84960877127?origin=scopusAI

38. Systematic narrative review of pediatric-to-adult care transition models for youth with pediatric-onset chronic conditions Marani, H., Fujioka, J., Tabatabavakili, S., Bollegala, N. Children and Youth Services Review, 2020 https://www.scopus.com/pages/publications/85090039794?origin=scopusAI

39. Performance of the ablation index during pulmonary vein isolation: periprocedural data from a multicenter registry Gasimova, N.Z., Nechepurenko, A.A., Kropotkin, E.B., (...), Mikhaylov, E.N. Journal of Interventional Cardiac Electrophysiology, 2022 https://www.scopus.com/pages/publications/85130119285?origin=scopusAI